

August 12, 2026

Humana
Provider Correspondence
P.O. Box 14601
Lexington, KY 40512

RE: Formal Appeal of Claim Denial — Louis Wisoky (MRN MUSC6784417), Claim MUSC-20260329-4577-3

Patient	Louis Wisoky (DOB 1938-04-09)
MRN	MUSC6784417
Member ID / Group	HUM416532893 / GRP-31881
Claim number	MUSC-20260329-4577-3
Date of service	2026-03-29
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$441.85
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-07-23

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health submits this formal first-level appeal on behalf of patient Louis Wisoky (MRN: MUSC6784417, Member ID: HUM416532893, Group: GRP-31881) regarding the denial of Claim MUSC-20260329-4577-3 for services rendered on March 29, 2026, at MUSC Health Ashley River Tower. Humana denied the claim on May 24, 2026, under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") and RARC N115 ("This decision was based on a Local Coverage Determination (LCD)"), with the accompanying remark that "Documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and requests immediate reversal and payment of the billed amount of \$441.85.

CLINICAL BACKGROUND

Mr. Wisoky is an 88-year-old male with a clinically complex, longitudinally documented medical history managed at MUSC Health. His active conditions include chronic obstructive bronchitis, chronic congestive heart failure, osteoporosis, gout, and a history of cardiac arrest. His medication regimen reflects this complexity and includes furosemide injection, metoprolol succinate extended release, Lasix, alendronic acid, and a fluticasone propionate/salmeterol combination dry powder inhaler that has been continuously prescribed since 1981. Spirometry results on record demonstrate a severely reduced FEV1/FVC ratio of 19.4%, and a left ventricular ejection fraction of 40.3% has been documented, consistent with his diagnosis of chronic congestive heart failure. A glomerular filtration rate of 20.3 mL/min further underscores the degree of systemic compromise this patient carries. The March 29, 2026, encounter was billed as CPT 99214, an established outpatient visit of moderate complexity, rendered by Dr. Sarah E. Whitmore, MD (NPI 1447382910), with a primary diagnosis of J06.9 (acute upper respiratory infection, unspecified) and a secondary diagnosis of R69.

BASIS FOR APPEAL

The denial is not supported by the clinical facts of this case. An acute upper respiratory infection in an 88-year-old patient with active chronic obstructive bronchitis, chronic congestive heart failure with reduced ejection fraction, and severely compromised renal function represents a clinically significant presentation that warrants prompt physician evaluation. In patients with this degree of underlying cardiopulmonary and renal disease, an acute respiratory illness carries meaningful risk of exacerbation, decompensation, and hospitalization. The selection of CPT 99214 for an established patient visit of moderate complexity is appropriate and well-supported given the number of active chronic conditions being managed, the complexity of the medication regimen, and the acute presentation layered upon that chronic burden. The payer's assertion that the documentation does not establish medical necessity fails to account for the totality of this patient's condition. Furthermore, RARC N115 references a Local Coverage Determination as the basis for denial; however, an LCD cannot be applied to override clinical judgment when the patient's documented comorbidities clearly elevate the medical necessity of the service in question. Humana is obligated under Medicare Advantage regulations to apply coverage standards no more restrictive than traditional Medicare, and traditional Medicare recognizes evaluation and management services for acute illness in high-risk established patients as reasonable and necessary.

REQUESTED ACTION

MUSC Health respectfully requests that Humana conduct a full clinical review of this appeal, including consideration of Mr. Wisoky's complete active problem list and medication history as documented herein. We request that the denial of Claim MUSC-20260329-4577-3 be reversed in full and that payment of \$441.85 be issued to MUSC Health promptly. Should Humana require additional clinical documentation to support this determination, please contact the MUSC Health Revenue Cycle Appeals team directly. This appeal is submitted within the applicable Medicare Advantage reconsideration deadline of July 23, 2026.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record